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PATIENT NAME : LOKESH KUMAR.S **REF. DOCTOR :** SELF

CODE/NAME & ADDRESS : C000161946 HWM SERVICES LIMITED LIABILITY PARTNERSHIP FLAT NO. 202, 2ND FLOOR, MANGAL BHAVAN APPARTMENT, KASHELI SHIVAJI MAIDAN,TALUKA THANE 421302 9892379050	ACCESSION NO : **0323ZJ006286** PATIENT ID : LOKEM060896323 CLNT.PATIENT ID: ABHA NO :	AGE/SEX :30 Years Male DRAWN :28/10/2026 13:08:17 Hrs RECEIVED : 28/10/2026 13:07:29 Hrs REPORTED :29/10/2026 11:04:40Hrs

Test Report Status **Final**	Results	Units
GUMS	HEALTHY	
SUMMARY		
RELEVANT HISTORY	NOT SIGNIFICANT	
RELEVANT GP EXAMINATION FINDINGS	NOT SIGNIFICANT	
RELEVANT LAB INVESTIGATIONS	URIC ACID IS ELEVATED. ELEVATED TOTAL CHOLESTEROL, CHOL:HDL RATIO ,LDL:HDL RATIO LEVEL NOTED .	
RELEVANT NON PATHOLOGY DIAGNOSTICS	NO ABNORMALITIES DETECTED	
REMARKS / RECOMMENDATIONS	URIC ACID IS ELEVATED. ADVISED TO REVIEW WITH PHYSICIAN. ADVISED TO AVOID ORGAN MEAT IF NON VEGETARIAN. ELEVATED TOTAL CHOLESTEROL, CHOL:HDL RATIO ,LDL:HDL RATIO LEVEL NOTED . ADVISED DIET AND LIFE STYLE MODIFICATION AND EXERCISE. ADVISED TO DO REVIEW WITH PHYSICIAN. USG ABDOMEN -GRADE I FATTY LIVER IS NOTED. ADVISED DIET AND LIFE STYLE MODIFICATION AND EXERCISE AND REVIEW WITH PHYSICIAN.	
FITNESS STATUS		
FITNESS STATUS	FIT (WITH MEDICAL ADVICE) (AS PER REQUESTED PANEL OF TESTS)	

Comments

OUR PANEL OF DOCTORS :

GENERAL PHYSICIANS - DR.MOHAMMED NAWAZ M.B.B.S

RADIOLOGIST - DR.SIVALINGAM.MD(RADIODIAGNOSIS),DMRD

SONOLOGIST -DR. UMALAKSHMI.G M.B.B.S

PULMONOGISTS - DR.M.VASANTHA KUMAR MBBS,DTCD

ENT PHYSICIANS - DR.NIRAJ JOSHI MBBS,MS,DLO.

OPHTHALMOLOGISTS - DR.K VASANTHA., MBBS,DO(OPHTHALMOLOGIST).

CARDIOLOGISTS - DR ARUN R M.D;DM

THIS REPORT CARRIES THE SIGNATURE OF OUR LABORATORY HEAD.
THIS IS AN INVIOLABLE FEATURE OF OUR LAB MANAGEMENT SOFTWARE.
HOWEVER ALL EXAMINATIONS AND INVESTIGATIONS HAVE BEEN CONDUCTED BY OUR PANEL OF DOCTORS.

Dr.Gayathri K
Consultant Pathologist

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Chennai, 600083
Tamil Nadu, India

ULR No.775000020711007-0323

DIAGNOSTIC REPORT

MC-5718

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PATIENT NAME : LOKESH KUMAR.S **REF. DOCTOR :** SELF

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Test Report Status **Final**	Results	Biological Reference Interval	Units

ENDOCRINOLOGY

PRE-EMPLOYMENT PACKAGE 3 MALE

OPIATES - URINE

OPIATES	NOT DETECTED	NOT DETECTED

AMPHETAMINE - URINE

AMPHETAMINE - URINE	NOT DETECTED	NOT DETECTED

Interpretation(s)
OPIATES - URINE-Lateral Flow Chromatography Immunoassay.

This is only a preliminary screening test. A more specific alternate chemical method must be used in order to obtain a confirmed analytical result. Gas chromatography/ mass spectometry or high performance liquid chromatography are preferred confirmatory methods.

****

CUT OFF LEVELS FOR DRUGS OF ABUSE IN URINE

ANALYTE	CUT OFF(AS PER KIT INSERT) ng/ml
OPIATES	2000

****

Kindly exert caution while interpreting the result for Opiates, as presence of Oxycodone (>1000 ng/mL) in the sample tends to give a false positive response due to structural similarity between Oxycodone and Codiene.
AMPHETAMINE - URINE-Lateral Flow Chromatography Immunoassay.

This is only a preliminary screening test. A more specific alternate chemical method must be used in order to obtain a confirmed analytical result. Gas chromatography/ mass spectometry or high performance liquid chromatography are preferred confirmatory methods.

****

CUT OFF LEVELS FOR DRUGS OF ABUSE IN URINE

ANALYTE	CUT OFF(AS PER KIT INSERT) ng/ml
AMPHETAMINE (AMP)	1000

****

Dr. Deepak Sanghavi,M.D(Path)
(Reg.no.MMC2004/03/1530)
Chief of Mumbai Reference Lab.

Dr.Hinal H.Shah MD,
(Biochemist)
(Reg.no.MMC2017/09/4172)
Consultant Biochemist

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CAP ACCREDITED COLLEGE of AMERICAN PATHOLOGISTS

ULR No.775000020711007-0002

PATIENT NAME : LOKESH KUMAR.S **REF. DOCTOR :** SELF

CODE/NAME & ADDRESS : C000161946 HWM SERVICES LIMITED LIABILITY PARTNERSHIP FLAT NO. 202, 2ND FLOOR, MANGAL BHAVAN APPARTMENT, KASHELI SHIVAJI MAIDAN,TALUKA THANE 421302 9892379050	ACCESSION NO : **0323ZJ006286** PATIENT ID : LOKEM060896323 CLNT.PATIENT ID: ABHA NO :	AGE/SEX :30 Years Male DRAWN :28/10/2026 13:08:17 Hrs RECEIVED : 28/10/2026 13:07:29 Hrs REPORTED :28/10/2026 15:54:48Hrs

Test Report Status **Final**	Results		Units
SPECIALISED CHEMISTRY - DRUG OF ABUSE			
PRE-EMPLOYMENT PACKAGE 3 MALE			
URINE - PCP (PHENCYCLIDINE), URINE			
PCP	NOT DETECTED	NOT DETECTED	
COCAINE - URINE			
COCAINE - URINE	NOT DETECTED	NOT DETECTED	
CANNABINOIDS - URINE			
CANNABINOIDS - URINE	NOT DETECTED	NOT DETECTED	
CREATININE, URINE	43	24 - 392	mg/dL

METHOD : SPECTROPHOTOMETRY, JAFFE'S ALKALINE PICRATE KINETIC - RATE BLANKED - IFCC-IDMS STANDARIZED

Interpretation(s)
URINE - PCP (PHENCYCLIDINE), URINE-Lateral Flow Chromatography Immunoassay.

Phencyclidine (PCP) is a popular drug of abuse. Its duration of effect is 2-4 hours. Its detection limit is (cut off) 25 ng/mL.

This is only a preliminary screening test. A more specific alternate chemical method must be used in order to obtain a confirmed analytical result. Gas chromatography/ mass spectometry or high performance liquid chromatography are preferred confirmatory methods.

****

CUT OFF LEVELS FOR DRUGS OF ABUSE IN URINE

ANALYTE	CUT OFF(AS PER KIT INSERT)
.	ng/ml
.	
PHENCYCLIDINE(PCP)	25

****

COCAINE - URINE-Lateral Flow Chromatography Immunoassay.

Cocaine is a nervous system stimulant that can be addictive. Cocaine may appear in Urine for only a few hours after use. Its detection limit is (cut off) is 300 ng/mL.

This is only a preliminary screening test. A more specific alternate chemical method must be used in order to obtain a confirmed analytical result. Gas chromatography/ mass spectometry or high performance liquid chromatography are preferred confirmatory methods.

****

CUT OFF LEVELS FOR DRUGS OF ABUSE IN URINE

ANALYTE	CUT OFF(AS PER KIT INSERT)
.	ng/ml
.	
COCAINE(C0C)	300

****

CANNABINOIDS - URINE-Lateral Flow Chromatography Immunoassay.

This is only a preliminary screening test. A more specific alternate chemical method must be used in order to obtain a confirmed analytical result. Gas chromatography/ mass spectometry or high performance liquid chromatography are preferred confirmatory methods.

****

CUT OFF LEVELS FOR DRUGS OF ABUSE IN URINE

ANALYTE	CUT OFF(AS PER KIT INSERT)
.	ng/ml
.	
CANNABINOIDS(THC)	50

****

Dr.Hinal H.Shah MD, (Biochemist) (Reg.no.MMC2017/09/4172) Consultant Biochemist

Dr. Deepak Sanghavi,M.D(Path) (Reg.no.MMC2004/03/1530) Chief of Mumbai Reference Lab.

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ULR No.775000020711007-0002

MC-7023

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PATIENT NAME : LOKESH KUMAR.S **REF. DOCTOR :** SELF

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Test Report Status **Final**	Results	Units
PRE-EMPLOYMENT PACKAGE 3 MALE		
BASIC EYE EXAMINATION		
CONJUNCTIVA	NORMAL	
EYELIDS	NORMAL	
EYE MOVEMENTS	NORMAL	
CORNEA	NORMAL	
DISTANT VISION RIGHT EYE WITHOUT GLASSES	WITHIN NORMAL LIMIT	
DISTANT VISION LEFT EYE WITHOUT GLASSES	WITHIN NORMAL LIMIT	
NEAR VISION RIGHT EYE WITHOUT GLASSES	WITHIN NORMAL LIMIT	
NEAR VISION LEFT EYE WITHOUT GLASSES	WITHIN NORMAL LIMIT	
COLOUR VISION	NORMAL	

Dr.Gayathri K
Consultant Pathologist

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ULR No.775000020711007-0323

DIAGNOSTIC REPORT

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Test Report Status **Final**	Results		Units
PRE-EMPLOYMENT PACKAGE 3 MALE			
ECG			
ECG	WITHIN NORMAL LIMITS		
XRAY-CHEST			
IMPRESSION	NO ABNORMALITY DETECTED		
AUDIOMETRY BASIC			
AUDIOMETRY	HEARING WITHIN NORMAL LIMITS		
LUNG FUNCTION TEST			
LUNG FUNCTION TEST	WITHIN NORMAL LIMITS		
MEDICAL HISTORY			
RELEVANT PRESENT HISTORY	NOT SIGNIFICANT		
RELEVANT PAST HISTORY	NOT SIGNIFICANT		
RELEVANT PERSONAL HISTORY	NOT SIGNIFICANT		
RELEVANT FAMILY HISTORY	SYSTEMIC HYPERTENSION+DIABETES MELLITUS-FATHER		
OCCUPATIONAL HISTORY	NOT SIGNIFICANT		
HISTORY OF MEDICATIONS	NOT SIGNIFICANT		
ANTHROPOMETRIC DATA & BMI			
HEIGHT IN METERS	1.75		mts
WEIGHT IN KGS.	76		Kgs
BMI	25	BMI & Weight Status as follows: Below 18.5: Underweight 18.5 - 24.9: Normal 25.0 - 29.9: Overweight 30.0 and Above: Obese	kg/sqmts
GENERAL EXAMINATION			
MENTAL / EMOTIONAL STATE	NORMAL		
PHYSICAL ATTITUDE	NORMAL		
GENERAL APPEARANCE / NUTRITIONAL STATUS	HEALTHY		
BUILT / SKELETAL FRAMEWORK	AVERAGE		
FACIAL APPEARANCE	NORMAL		
SKIN	NORMAL		
UPPER LIMB	NORMAL		
LOWER LIMB	NORMAL		
NECK	NORMAL		

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Test Report Status **Final**	Results	Units
NECK LYMPHATICS / SALIVARY GLANDS	NOT ENLARGED OR TENDER	
THYROID GLAND	NOT ENLARGED	
CAROTID PULSATION	NORMAL	
TEMPERATURE	98.4 F	
PULSE	74/MIN	
RESPIRATORY RATE	16/MIN	
CARDIOVASCULAR SYSTEM		
BP	128/74	mm/Hg
PERICARDIUM	NORMAL	
APEX BEAT	NORMAL	
HEART SOUNDS	S1, S2 HEARD NORMALLY	
MURMURS	ABSENT	
RESPIRATORY SYSTEM		
SIZE AND SHAPE OF CHEST	NORMAL	
MOVEMENTS OF CHEST	SYMMETRICAL	
BREATH SOUNDS INTENSITY	NORMAL	
BREATH SOUNDS QUALITY	VESICULAR (NORMAL)	
ADDED SOUNDS	ABSENT	
PER ABDOMEN		
APPEARANCE	NORMAL	
VENOUS PROMINENCE	ABSENT	
LIVER	NOT PALPABLE	
SPLEEN	NOT PALPABLE	
HERNIA	ABSENT	
BASIC ENT EXAMINATION		
EXTERNAL EAR CANAL	NORMAL	
TYMPANIC MEMBRANE	NORMAL	
NOSE	NO ABNORMALITY DETECTED	
SINUSES	NORMAL	
THROAT	NO ABNORMALITY DETECTED	
TONSILS	NOT ENLARGED	
BASIC DENTAL EXAMINATION		
TEETH	NORMAL	

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Test Report Status **Final**	Results	Units

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PRE-EMPLOYMENT PACKAGE 3 MALE

ULTRASOUND ABDOMEN

ULTRASOUND ABDOMEN

* GRADE I FATTY LIVER.

Interpretation(s)
MEDICAL
HISTORY-**********
THIS REPORT CARRIES THE SIGNATURE OF OUR LABORATORY DIRECTOR. THIS IS AN INVIOLABLE FEATURE OF OUR LAB MANAGEMENT SOFTWARE. HOWEVER, ALL EXAMINATIONS AND INVESTIGATIONS HAVE BEEN CONDUCTED BY OUR PANEL OF DOCTORS.

****
FITNESS STATUS-Conclusion on an individual's Fitness, which is commented upon mainly for Pre employment cases, is based on multi factorial findings and does not depend on any one single parameter. The final Fitness assigned to a candidate will depend on the Physician's findings and overall judgement on a case to case basis, details of the candidate's past and personal history; as well as the comprehensiveness of the diagnostic panel which has been requested for .These are then further correlated with details of the job under consideration to eventually fit the right man to the right job.
Basis the above, Agilus diagnostic classifies a candidate's Fitness Status into one of the following categories:
• Fit (As per requested panel of tests) – AGILUS Limited gives the individual a clean chit to join the organization, on the basis of the General Physical Examination and the specific test panel requested for.
• Fit (with medical advice) (As per requested panel of tests) - This indicates that although the candidate can be declared as FIT to join the job, minimal problems have been detected during the Pre- employment examination. Examples of conditions which could fall in this category could be cases of mild reversible medical abnormalities such as height weight disproportions, borderline raised Blood Pressure readings, mildly raised Blood sugar and Blood Lipid levels, Hematuria, etc. Most of these relate to sedentary lifestyles and come under the broad category of life style disorders. The idea is to caution an individual to bring about certain lifestyle changes as well as seek a Physician''''''

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Test Report Status **Final**	Results	Units
HAEMATOLOGY - CBC		
PRE-EMPLOYMENT PACKAGE 3 MALE		
BLOOD COUNTS,EDTA WHOLE BLOOD		
HEMOGLOBIN (HB) METHOD : SLS METHOD	14.0	13.0 - 17.0 g/dL
RED BLOOD CELL (RBC) COUNT	4.60	4.5 - 5.5 mil/µL
WHITE BLOOD CELL (WBC) COUNT	7.40	4.0 - 10.0 thou/µL
PLATELET COUNT	321	150 - 410 thou/µL
RBC AND PLATELET INDICES		
HEMATOCRIT (PCV) METHOD : PULSE HEIGHT DETECTION	42.2	40.0 - 50.0 %
MEAN CORPUSCULAR VOLUME (MCV) METHOD : CALCULATED PARAMETER	91.7	83.0 - 101.0 fL
MEAN CORPUSCULAR HEMOGLOBIN (MCH)	30.4	27.0 - 32.0 pg
MEAN CORPUSCULAR HEMOGLOBIN CONCENTRATION (MCHC)	33.2	31.5 - 34.5 g/dL
RED CELL DISTRIBUTION WIDTH (RDW)	**14.3 High**	11.6 - 14.0 %
MEAN PLATELET VOLUME (MPV)	8.5	6.8 - 10.9 fL
WBC DIFFERENTIAL COUNT		
NEUTROPHILS	54	40 - 80 %
LYMPHOCYTES	37	20 - 40 %
MONOCYTES METHOD : FLOWCYTOMETRY	6	2 - 10 %
EOSINOPHILS	3	1 - 6 %
BASOPHILS	0	0 - 1 %
ABSOLUTE NEUTROPHIL COUNT METHOD : AUTOMATED CELL COUNTER : CALCULATED PARAMETER	3.97	2.0 - 7.0 thou/µL
ABSOLUTE LYMPHOCYTE COUNT METHOD : AUTOMATED CELL COUNTER : CALCULATED PARAMETER	2.73	1.0 - 3.0 thou/µL
ABSOLUTE MONOCYTE COUNT METHOD : AUTOMATED CELL COUNTER : CALCULATED PARAMETER	0.44	0.2 - 1.0 thou/µL
ABSOLUTE EOSINOPHIL COUNT METHOD : AUTOMATED CELL COUNTER : CALCULATED PARAMETER	0.24	0.02 - 0.50 thou/µL
ABSOLUTE BASOPHIL COUNT	0.02	0.0 - 0.1 thou/µL
PS(PERIPHERAL SMEAR EXAM,EDTA WHOLE BLOOD		
RBC METHOD : LEISHMAN STAINING / MICROSCOPY	PREDOMINANTLY NORMOCYTIC NORMOCHROMIC	

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DIAGNOSTIC REPORT

MC-7023

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Test Report Status **Final**	**Results**	**Units**
WBC METHOD : LEISHMAN STAINING / MICROSCOPY	WBCS ARE NORMAL IN NUMBER & MORPHOLOGY	
PLATELETS METHOD : LEISHMAN STAINING / MICROSCOPY	ADEQUATE	
IMPRESSION METHOD : LEISHMAN STAINING / MICROSCOPY	NORMAL SMEAR STUDY.	

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Test Report Status **Final**	Results	Units

PRE-EMPLOYMENT PACKAGE 3 MALE

ERYTHROCYTE SEDIMENTATION RATE (ESR),WHOLE BLOOD

E.S.R	**25 High**	0 - 10	mm at 1 hr

Interpretation(s)

BLOOD COUNTS,EDTA WHOLE BLOOD-The cell morphology is well preserved for 24hrs. However after 24-48 hrs a progressive increase in MCV and HCT is observed leading to a decrease in MCHC. A direct smear is recommended for an accurate differential count and for examination of RBC morphology.
RBC AND PLATELET INDICES-Mentzer index (MCV/RBC) is an automated cell-counter based calculated screen tool to differentiate cases of Iron deficiency anaemia(>13) from Beta thalassaemia trait (<13) in patients with microcytic anaemia. This needs to be interpreted in line with clinical correlation and suspicion. Estimation of HbA2 remains the gold standard for diagnosing a case of beta thalassaemia trait.
WBC DIFFERENTIAL COUNT-The optimal threshold of 3.3 for NLR showed a prognostic possibility of clinical symptoms to change from mild to severe in COVID positive patients. When age = 49.5 years old and NLR = 3.3, 46.1% COVID-19 patients with mild disease might become severe. By contrast, when age < 49.5 years old and NLR < 3.3, COVID-19 patients tend to show mild disease.
(Reference to - The diagnostic and predictive role of NLR, d-NLR and PLR in COVID-19 patients ; A.-P. Yang, et al.; International Immunopharmacology 84 (2020) 106504
This ratio element is a calculated parameter and out of NABL scope.
ERYTHROCYTE SEDIMENTATION RATE (ESR),WHOLE BLOOD-**TEST DESCRIPTION** :- Erythrocyte sedimentation rate (ESR) is a test that indirectly measures the degree of inflammation present in the body. The test actually measures the rate of fall (sedimentation) of erythrocytes in a sample of blood that has been placed into a tall, thin, vertical tube. Results are reported as the millimetres of clear fluid (plasma) that are present at the top portion of the tube after one hour. Nowadays fully automated instruments are available to measure ESR.
- ESR is not diagnostic; it is a non-specific test that may be elevated in a number of different conditions. It provides general information about the presence of an inflammatory condition.CRP is superior to ESR because it is more sensitive and reflects a more rapid change.
TEST INTERPRETATION : **Increase** in: Infections, Vasculities, Inflammatory arthritis, Renal disease, Anemia, Malignancies and plasma cell dyscrasias, Acute allergy Tissue injury, Pregnancy, Estrogen medication, Aging.
Finding a very accelerated ESR**(>100 mm/hour)** in patients with ill-defined symptoms directs the physician to search for a systemic disease (Paraproteinemias, Disseminated malignancies, connective tissue disease, severe infections such as bacterial endocarditis).
In pregnancy BRI in first trimester is 0-48 mm/hr(62 if anemic) and in second trimester (0-70 mm /hr(95 if anemic). ESR returns to normal 4th week post partum.
Decreased in: Polycythermia vera, Sickle cell anemia
LIMITATIONS : **False elevated** ESR : Increased fibrinogen, Drugs(Vitamin A, Dextran etc), Hypercholesterolemia
False Decreased : Poikilocytosis,(SickleCells,spherocytes),Microcytosis, Low fibrinogen, Very high WBC counts, Drugs(Quinine, salicylates)
REFERENCE : Nathan and Oski's Haematology of Infancy and Childhood, 5th edition;2. Paediatric reference intervals. AACC Press, 7th edition. Edited by S. Soldin;3. The reference for the adult reference range is "Practical Haematology by Dacie and Lewis,10th edition.

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ULR No.775000020711007-0323

PATIENT NAME : LOKESH KUMAR.S **REF. DOCTOR :** SELF

CODE/NAME & ADDRESS : C000161946 HWM SERVICES LIMITED LIABILITY PARTNERSHIP FLAT NO. 202, 2ND FLOOR, MANGAL BHAVAN APPARTMENT, KASHELI SHIVAJI MAIDAN,TALUKA THANE 421302 9892379050	ACCESSION NO : **0323ZJ006286** PATIENT ID : LOKEM060896323 CLNT.PATIENT ID: ABHA NO :	AGE/SEX :30 Years Male DRAWN :28/10/2026 13:08:17 Hrs RECEIVED : 28/10/2026 13:07:29 Hrs REPORTED :28/10/2026 18:16:52Hrs

Test Report Status **Final**	Results	Units

HAEMATOLOGY

PRE-EMPLOYMENT PACKAGE 3 MALE

GLYCOSYLATED HEMOGLOBIN(HBA1C),EDTA WHOLE BLOOD

HBA1C	5.1	Non-diabetic: < 5.7 Pre-diabetics: 5.7 - 6.4 Diabetics: > or = 6.5 Therapeutic goals: < 7.0 Action suggested : > 8.0 (ADA Guideline 2021)	%
METHOD : HIGH PERFORMANCE LIQUID CHROMATOGRAPHY (HPLC)			
ESTIMATED AVERAGE GLUCOSE(EAG)	99.7	< 116.0	mg/dL
METHOD : CALCULATED PARAMETER			

Interpretation(s)
GLYCOSYLATED HEMOGLOBIN(HBA1C),EDTA WHOLE BLOOD-Glycosylated hemoglobin (GHb) has been firmly established as an index of long-term blood glucose concentrations and as a measure of the risk for the development of complications in patients with diabetes mellitus. Formation of GHb is essentially irreversible, and the concentration in the blood depends on both the life span of the red blood cell (average 120 days) and the blood glucose concentration. Because the rate of formation of GHb is directly proportional to the concentration of glucose in the blood, the GHb concentration represents the integrated values for glucose over the preceding 6-8 weeks.
Any condition that alters the life span of the red blood cells has the potential to alter the GHb level. Samples from patients with hemolytic anemias will exhibit decreased glycated hemoglobin values due to the shortened life span of the red cells. This effect will depend upon the severity of the anemia. Samples from patients with polycythemia or post-splenectomy may exhibit increased glycated hemoglobin values due to a somewhat longer life span of the red cells.
Glycosylated hemoglobins results from patients with HbSS, HbCC, and HbSC and HbD must be interpreted with caution, given the pathological processes, including anemia, increased red cell turnover, transfusion requirements, that adversely impact HbA1c as a marker of long-term glycemic control. In these conditions, alternative forms of testing such as glycated serum protein (fructosamine) should be considered.
References
1. Tietz Textbook of Clinical Chemistry and Molecular Diagnostics, edited by Carl A Burtis, Edward R.Ashwood, David E Bruns, 4th Edition, Elsevier publication, 2006, 879-884.
2. Forsham PH. Diabetes Mellitus:A rational plan for management. Postgrad Med 1982, 71,139-154.
3. Mayer TK, Freedman ZR: Protein glycosylation in Diabetes Mellitus: A review of laboratory measurements and their clinical utility. Clin Chim Acta 1983, 127, 147-184.

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Test Report Status **Final**	Results	Units

IMMUNOHAEMATOLOGY

PRE-EMPLOYMENT PACKAGE 3 MALE

ABO GROUP & RH TYPE, EDTA WHOLE BLOOD

ABO GROUP	B	
METHOD : HAEMAGGLUTINATION (AUTOMATED)		
RH TYPE	Negative	
METHOD : HAEMAGGLUTINATION (AUTOMATED)		

Interpretation(s)
ABO GROUP & RH TYPE, EDTA WHOLE BLOOD-Blood group is identified by antigens and antibodies present in the blood. Antigens are protein molecules found on the surface of red blood cells. Antibodies are found in plasma. To determine blood group, red cells are mixed with different antibody solutions to give A,B,O or AB.

Disclaimer: "Please note, as the results of previous ABO and Rh group (Blood Group) for pregnant women are not available, please check with the patient records for availability of the same."

The test is performed by both forward as well as reverse grouping methods.

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BIOCHEMISTRY		
PRE-EMPLOYMENT PACKAGE 3 MALE		
GLUCOSE FASTING (FBS), FLUORIDE PLASMA		
FBS-FASTING BLOOD SUGAR(GLUCOSE)	84	(Normal <100,Impaired fasting glucose:100 to 125,Diabetes mellitus:>=126(on more than 1 occasion)(ADA guidelines 2024) mg/dL
METHOD : HEXOKINASE		
BLOOD UREA NITROGEN (BUN), SERUM		
BLOOD UREA NITROGEN	8	6 - 20 mg/dL
METHOD : UREASE -GLDH		
CREATININE, SERUM		
CREATININE	**0.82 Low**	0.90 - 1.30 mg/dL
METHOD : ALKALINE PICRATE (MODIFIED JAFFE REACTION)		
LIVER FUNCTION PROFILE, SERUM		
BILIRUBIN, TOTAL	0.52	Upto 1.2 mg/dL
METHOD : DIAZO METHOD FOLLOWED BY SPECTROPHOTOMETERY		
BILIRUBIN, DIRECT	0.24	0.0 - 0.3 mg/dL
METHOD : DIAZO METHOD		
BILIRUBIN, INDIRECT	0.28	0.0 - 0.9 mg/dL
METHOD : CALCULATED PARAMETER		
TOTAL PROTEIN	7.5	6.0 - 8.0 g/dL
METHOD : BIURET		
ALBUMIN	4.9	3.5 - 5.2 g/dL
METHOD : BROMOCRESOL GREEN		
GLOBULIN	2.6	1.5 - 3.5 g/dL
METHOD : CALCULATED		
ALBUMIN/GLOBULIN RATIO	1.9	0.90 - 2.00 RATIO
METHOD : CALCULATED PARAMETER		
ASPARTATE AMINOTRANSFERASE(AST/SGOT)	19	< 35 U/L
METHOD : UV KINETIC WITHOUT PYRIDOXAL-5 -PHOSPHATE - IFCC		
ALANINE AMINOTRANSFERASE (ALT/SGPT)	18	<45 U/L
METHOD : UV KINETIC WITHOUT PYRIDOXAL-5 -PHOSPHATE - IFCC		
ALKALINE PHOSPHATASE	82	40 - 129 U/L
METHOD : PNPP, AMP BUFFER-IFCC		
GAMMA GLUTAMYL TRANSFERASE (GGT)	19	< 55 U/L
METHOD : G-GLUTAMYL-CARBOXY-NITROANILIDE-IFCC		
LACTATE DEHYDROGENASE	183	135 - 225 U/L
METHOD : IFCC LACTATE TO PYRUVATE		

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Test Report Status **Final**	Results		Units
PRE-EMPLOYMENT PACKAGE 3 MALE			
URIC ACID, SERUM			
URIC ACID	**9.1 High**	3.5-7.2	mg/dL
METHOD : SPECTROPHOTOMETRY, URICASE			

Interpretation(s)
GLUCOSE FASTING (FBS), FLUORIDE PLASMA-**TEST DESCRIPTION**
Normally, the glucose concentration in extracellular fluid is closely regulated so that a source of energy is readily available to tissues and sothat no glucose is excreted in the urine.
Increased in:Diabetes mellitus, Cushing' s syndrome (10 – 15%), chronic pancreatitis (30%). Drugs:corticosteroids,phenytoin, estrogen, thiazides.
Decreased in :Pancreatic islet cell disease with increased insulin,insulinoma,adrenocortical insufficiency,hypopituitarism,diffuse liver disease, malignancy(adrenocortical,stomach,fibrosarcoma),infant of a diabetic mother,enzyme deficiency diseases(e.g.galactosemia),Drugs-insulin,ethanol,propranolol;sulfonylureas,tolbutamide,and other oral hypoglycemic agents.
NOTE: While random serum glucose levels correlate with home glucose monitoring results (weekly mean capillary glucose values),there is wide fluctuation within individuals.Thus, glycosylated hemoglobin(HbA1c) levels are favored to monitor glycemic control.
High fasting glucose level in comparison to post prandial glucose level may be seen due to effect of Oral Hypoglycaemics & Insulin treatment,Renal Glyosuria,Glycaemic index & response to food consumed,Alimentary Hypoglycemia,Increased insulin response & sensitivity etc.

Glucose fasting, plasma as per ADA Guideline
NORMAL RANGE: <100 mg/dl
PRE-DIABETIC RANGE: 100-125 mg/dl (Impaired Fasting Glucose)
DIABETIC RANGE: = 126 mg/dl
BLOOD UREA NITROGEN (BUN), SERUM-**Causes of Increased** levels include Pre renal (High protein diet, Increased protein catabolism, GI haemorrhage, Cortisol, Dehydration, CHF Renal), Renal Failure, Post Renal (Malignancy, Nephrolithiasis, Prostatism)
Causes of decreased level include Liver disease, SIADH.
CREATININE, SERUM-**Higher than normal level may be due to:**
• Blockage in the urinary tract, Kidney problems, such as kidney damage or failure, infection, or reduced blood flow, Loss of body fluid (dehydration), Muscle problems, such as breakdown of muscle fibers, Problems during pregnancy, such as seizures (eclampsia)), or high blood pressure caused by pregnancy (preeclampsia)
Lower than normal level may be due to:• Myasthenia Gravis, Muscuophy
LIVER FUNCTION PROFILE, SERUM-**Bilirubin** is a yellowish pigment found in bile and is a breakdown product of normal heme catabolism. Bilirubin is excreted in bile and urine, and elevated levels may give yellow discoloration in jaundice.**Elevated levels** results from increased bilirubin production (eg, hemolysis and ineffective erythropoiesis), decreased bilirubin excretion (eg, obstruction and hepatitis), and abnormal bilirubin metabolism (eg, hereditary and neonatal jaundice). Conjugated (direct) bilirubin is elevated more than unconjugated (indirect) bilirubin in Viral hepatitis, Drug reactions, Alcoholic liver disease Conjugated (direct) bilirubin is also elevated more than unconjugated (indirect) bilirubin when there is some kind of blockage of the bile ducts like in Gallstones getting into the bile ducts, tumors &Scarring of the bile ducts. Increased unconjugated (indirect) bilirubin may be a result of Hemolytic or pernicious anemia, Transfusion reaction & a common metabolic condition termed Gilbert syndrome, due to low levels of the enzyme that attaches sugar molecules to bilirubin.
AST is an enzyme found in various parts of the body. AST is found in the liver, heart, skeletal muscle, kidneys, brain, and red blood cells, and it is commonly measured clinically as a marker for liver health. AST levels increase during chronic viral hepatitis, blockage of the bile duct, cirrhosis of the liver,liver cancer,kidney failure,hemolytic anemia,pancreatitis,hemochromatosis. AST levels may also increase after a heart attack or strenuous activity.ALT test measures the amount of this enzyme in the blood.ALT is found mainly in the liver, but also in smaller amounts in the kidneys,heart,muscles, and pancreas.It is commonly measured as a part of a diagnostic evaluation of hepatocellular injury, to determine liver health.AST levels increase during acute hepatitis,sometimes due to a viral infection,ischemia to the liver,chronic hepatitis,obstruction of bile ducts,cirrhosis.
ALP is a protein found in almost all body tissues.Tissues with higher amounts of ALP include the liver,bile ducts and bone.Elevated ALP levels are seen in Biliary obstruction, Osteoblastic bone tumors, osteomalacia, hepatitis, Hyperparathyroidism, Leukemia, Lymphoma, Pagets disease,Rickets,Sarcoidosis etc. Lower-than-normal ALP levels seen in Hypophosphatasia,Malnutrition,Protein deficiency,Wilsons disease.
GGT is an enzyme found in cell membranes of many tissues mainly in the liver,kidney and pancreas.It is also found in other tissues including intestine,spleen,heart, brain and seminal vesicles.The highest concentration is in the kidney,but the liver is considered the source of normal enzyme activity.Serum GGT has been widely used as an index of liver dysfunction.Elevated serum GGT activity can be found in diseases of the liver,biliary system and pancreas.Conditions that increase serum GGT are obstructive liver disease,high alcohol consumption and use of enzyme-inducing drugs etc.
Total Protein also known as total protein,is a biochemical test for measuring the total amount of protein in serum.Protein in the plasma is made up of albumin and globulin.Higher-than-normal levels may be due to:Chronic inflammation or infection,including HIV and hepatitis B or C,Multiple myeloma,Waldenstroms disease.Lower-than-normal levels may be due to: Agammaglobulinemia,Bleeding (hemorrhage),Burns,Glomerulonephritis,Liver disease, Malabsorption,Malnutrition,Nephrotic syndrome,Protein-losing enteropathy etc.
Albumin is the most abundant protein in human blood plasma.It is produced in the liver.Albumin constitutes about half of the blood serum protein.Low blood albumin levels (hypoalbuminemia) can be caused by:Liver disease like cirrhosis of the liver, nephrotic syndrome,protein-losing enteropathy,Burns,hemodilution,increased vascular permeability or decreased lymphatic clearance,malnutrition and wasting etc
URIC ACID, SERUM-**Causes of Increased levels:**-Dietary(High Protein Intake,Prolonged Fasting,Rapid weight loss),Gout,Lesch nyhan syndrome,Type 2 DM,Metabolic syndrome **Causes of decreased levels**-Low Zinc intake,OCP,Multiple Sclerosis

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Test Report Status **Final**	Results	Units

BIOCHEMISTRY - LIPID

PRE-EMPLOYMENT PACKAGE 3 MALE

LIPID PROFILE WITH CALCULATED LDL,SERUM

CHOLESTEROL, TOTAL	**182 High**	<170 0-18yrs: < 170 Desirable, 170-199 Borderline High, >= 200 High 18-120yrs: Desirable: <200, Borderline high cholesterol: 200-239, High cholesterol: >240	mg/dL
METHOD : CHOD - PAP			
TRIGLYCERIDES	141	< 150 Desirable, 150-199 Borderline High >200 High >500 Very High	mg/dL
METHOD : ENZYMATIC, END POINT			
HDL CHOLESTEROL	**35 Low**	Low: <40 High >/=60	mg/dL
METHOD : HOMOGENOUS DIRECT ENZYMATIC COLORIMETRIC			
CHOLESTEROL LDL	119	Optimal<100 Near Optimal 100-129 Borderline High 130-159 High 160-189 Very high >190	mg/dL
METHOD : CALCULATED PARAMETER			
NON HDL CHOLESTEROL	147	Optimal <130 Desirable 130-159 Borderline High 160-189 High 190-219 Very high >/=220	mg/dL
METHOD : CALCULATED PARAMETER			
VERY LOW DENSITY LIPOPROTEIN	28.2	< /= 30	mg/dL
CHOL/HDL RATIO	**5.2 High**	Optimal < 3.5 Desirable 3.5 - 5.0 High Risk > 5.0	
LDL/HDL RATIO	**3.4 High**	Desirable/Low Risk: 0.5 -3.0 Borderline/Moderate Risk: 3.1 – 6.0 High Risk: > 6.0	
METHOD : CALCULATED PARAMETER			

Interpretation(s)

Serum lipid profile is measured for cardiovascular risk prediction. Lipid Association of India recommends LDL-C as primary target and Non

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HDL-C as co-primary treatment target.

Risk Stratification for ASCVD (Atherosclerotic cardiovascular disease) by Lipid Association of India

Risk Category	
Extreme risk group	A.CAD with > 1 feature of high risk group
	B. CAD with > 1 feature of Very high risk group or recurrent ACS (within 1 year) despite LDL-C < or = 50 mg/dl or polyvascular disease
Very High Risk	1. Established ASCVD 2. Diabetes with 2 major risk factors or evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia
High Risk	1. Three major ASCVD risk factors. 2. Diabetes with 1 major risk factor or no evidence of end organ damage. 3. CKD stage 3B or 4. 4. LDL >190 mg/dl 5. Extreme of a single risk factor. 6. Coronary Artery Calcium - CAC >300 AU. 7. Lipoprotein a >/= 50mg/dl 8. Non stenotic carotid plaque
Moderate Risk	2 major ASCVD risk factors
Low Risk	0-1 major ASCVD risk factors
Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors	
1. Age > or = 45 years in males and > or = 55 years in females	3. Current Cigarette smoking or tobacco use
2. Family history of premature ASCVD	4. High blood pressure
5. Low HDL	

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by LAI in 2020.

Risk Group	**Treatment Goals**		**Consider Drug Therapy**	
	LDL-C (mg/dl)	**Non-HDL (mg/dl)**	**LDL-C (mg/dl)**	**Non-HDL (mg/dl)**
Extreme Risk Group Category A	<50 (Optional goal < OR = 30)	< 80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	<OR = 30	<OR = 60	> 30	>60
Very High Risk	<50	<80	>OR= 50	>OR= 80
High Risk	<70	<100	>OR= 70	>OR= 100
Moderate Risk	<100	<130	>OR= 100	>OR= 130
Low Risk	<100	<130	>OR= 130*	>OR= 160

*After an adequate non-pharmacological intervention for at least 3 months.

References: Management of Dyslipidaemia for the Prevention of Stroke: Clinical Practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology, 2022, 20, 134-155.

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Test Report Status **Final**	Results	Units

CLINICAL PATH - URINALYSIS

PRE-EMPLOYMENT PACKAGE 3 MALE

PHYSICAL EXAMINATION, URINE

COLOR METHOD : GROSS EXAMINATION	PALE YELLOW		
APPEARANCE METHOD : GROSS EXAMINATION	CLEAR		

CHEMICAL EXAMINATION, URINE

PH METHOD : PHOTOELECTRIC COLORIMETRY	5.0	4.5 - 7.5	
SPECIFIC GRAVITY METHOD : PHOTOELECTRIC COLORIMETRY	1.010	1.005 - 1.030	
PROTEIN METHOD : PHOTOELECTRIC COLORIMETRY	NOT DETECTED	NOT DETECTED	
GLUCOSE METHOD : PHOTOELECTRIC COLORIMETRY	NOT DETECTED	NOT DETECTED	
KETONES METHOD : PHOTOELECTRIC COLORIMETRY	NOT DETECTED	NOT DETECTED	
BLOOD METHOD : PHOTOELECTRIC COLORIMETRY	NOT DETECTED	NOT DETECTED	
BILIRUBIN METHOD : PHOTOELECTRIC COLORIMETRY/AZO COUPLING METHOD	NOT DETECTED	NOT DETECTED	
UROBILINOGEN METHOD : PHOTOELECTRIC COLORIMETRY	Normal		
NITRITE METHOD : PHOTOELECTRIC COLORIMETRY	NOT DETECTED	NOT DETECTED	
LEUKOCYTE ESTERASE METHOD : PHOTOELECTRIC COLORIMETRY	NOT DETECTED	NOT DETECTED	

MICROSCOPIC EXAMINATION, URINE

RED BLOOD CELLS METHOD : MICROSCOPY	NOT DETECTED	0 - 2	/HPF
PUS CELL (WBCS) METHOD : MICROSCOPY	2-3	0-5	/HPF
EPITHELIAL CELLS METHOD : MICROSCOPY	1-2	0-5	/HPF
CASTS METHOD : MICROSCOPY	NOT DETECTED		
CRYSTALS METHOD : MICROSCOPY	NOT DETECTED		

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BACTERIA	NOT DETECTED	NOT DETECTED
METHOD : MICROSCOPY		
YEAST	NOT DETECTED	NOT DETECTED
METHOD : MICROSCOPY		

Interpretation(s)

The following table describes the probable conditions, in which the analytes are present in urine

Presence of	Conditions
Proteins	Inflammation or immune illnesses
Pus (White Blood Cells)	Urinary tract infection, urinary tract or kidney stone, tumors or any kind of kidney impairment
Glucose	Diabetes or kidney disease
Ketones	Diabetic ketoacidosis (DKA), starvation or thirst
Urobilinogen	Liver disease such as hepatitis or cirrhosis
Blood	Renal or genital disorders/trauma
Bilirubin	Liver disease
Erythrocytes	Urological diseases (e.g. kidney and bladder cancer, urolithiasis), urinary tract infection and glomerular diseases
Leukocytes	Urinary tract infection, glomerulonephritis, interstitial nephritis either acute or chronic, polycystic kidney disease, urolithiasis, contamination by genital secretions
Epithelial cells	Urolithiasis, bladder carcinoma or hydronephrosis, ureteric stents or bladder catheters for prolonged periods of time
Granular Casts	Low intratubular pH, high urine osmolality and sodium concentration, interaction with Bence-Jones protein
Hyaline casts	Physical stress, fever, dehydration, acute congestive heart failure, renal diseases
Calcium oxalate	Metabolic stone disease, primary or secondary hyperoxaluria, intravenous infusion of large doses of vitamin C, the use of vasodilator naftidrofuryl oxalate or the gastrointestinal lipase inhibitor orlistat, ingestion of ethylene glycol or of star fruit (Averrhoa carambola) or its juice
Uric acid	arthritis
Bacteria	Urinary infectionwhen present in significant numbers & with pus cells.
Trichomonas vaginalis	Vaginitis, cervicitis or salpingitis

Reference:

Henrys Clinical Diagnostics and Management by Laboratory Methods 21st edition (pg 410).

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DIAGNOSTIC REPORT

agilus diagnostics
Formerly SRL

PATIENT NAME : LOKESH KUMAR.S | **REF. DOCTOR :** SELF

CODE/NAME & ADDRESS : C000161946 HWM SERVICES LIMITED LIABILITY PARTNERSHIP FLAT NO. 202, 2ND FLOOR, MANGAL BHAVAN APPARTMENT, KASHELI SHIVAJI MAIDAN,TALUKA THANE 421302 9892379050	ACCESSION NO : **0323ZJ006286** PATIENT ID : LOKEM060896323 CLNT.PATIENT ID: ABHA NO :	AGE/SEX :30 Years Male DRAWN :28/10/2026 13:08:17 Hrs RECEIVED : 28/10/2026 13:07:29 Hrs REPORTED :28/10/2026 14:57:24Hrs

Test Report Status **Final**	Results	Units

SPECIALISED CHEMISTRY - TUMOR MARKER

PRE-EMPLOYMENT PACKAGE 3 MALE

PROSTATE SPECIFIC ANTIGEN(PSA TOTAL), SERUM

PROSTATE SPECIFIC ANTIGEN	0.591	< 1.4	ng/mL

METHOD : ELECTROCHEMILUMINESCENCE

Interpretation(s)
PROSTATE SPECIFIC ANTIGEN(PSA TOTAL), SERUM-- PSA is detected in the male patients with normal, benign hyperplastic and malignant prostate tissue and in patients with prostatitis.
- PSA is not detected (or detected at very low levels) in the patients without prostate tissue (because of radical prostatectomy or cystoprostatectomy) and also in the female patients.
- It a suitable marker for monitoring of patients with Prostate Cancer and it is better to be used in conjunction with other diagnostic procedures.
- Serial PSA levels can help determine the success of prostatectomy and the need for further treatment, such as radiation, endocrine or chemotherapy and useful in detecting residual disease and early recurrence of tumor.
- Elevated levels of PSA can be also observed in the patients with non-malignant diseases like Prostatitis and Benign Prostatic Hyperplasia.
- Specimens for total PSA assay should be obtained before biopsy, prostatectomy or prostatic massage, since manipulation of the prostate gland may lead to elevated PSA (false positive) levels persisting up to 3 weeks.
- As per American urological guidelines, PSA screening is recommended for early detection of Prostate cancer above the age of 40 years. Following Age specific reference range can be used as a guide lines.
- Measurement of total PSA alone may not clearly distinguish between benign prostatic hyperplasia (BPH) from cancer, this is especially true for the total PSA values between 4-10 ng/mL.
- Total PSA values determined on patient samples by different testing procedures cannot be directly compared with one another and could be the cause of erroneous medical interpretations. Recommended follow up on same platform as patient result can vary due to differences in assay method and reagent specificity.

References-
1. Burtis CA, Ashwood ER, Bruns DE. Teitz textbook of clinical chemistry and Molecular Diagnostics. 4th edition.
2. Williamson MA, Snyder LM. Wallach's interpretation of diagnostic tests. 9th edition.

****End Of Report****
Please visit www.agilusdiagnostics.com for related Test Information for this accession

Dr.Gayathri K
Consultant Pathologist

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PERFORMED AT :
Agilus Diagnostics Ltd
Veda Ranghaa Nivas, No:22/97,Dr.Ambedkar Road, Ashok Nagar,4th Avenue
Chennai, 600083
Tamil Nadu, India

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CONDITIONS OF LABORATORY TESTING & REPORTING

1. It is presumed that the test sample belongs to the patient named or identified in the test requisition form.
2. All tests are performed and reported as per the turnaround time stated in the AGILUS Directory of Services.
3. Result delays could occur due to unforeseen circumstances such as non-availability of kits / equipment breakdown / natural calamities / technical downtime or any other unforeseen event.
4. A requested test might not be performed if:
   i. Specimen received is insufficient or inappropriate
   ii. Specimen quality is unsatisfactory
   iii. Incorrect specimen type
   iv. Discrepancy between identification on specimen container label and test requisition form
5. AGILUS Diagnostics confirms that all tests have been performed or assayed with highest quality standards, clinical safety & technical integrity.
6. Laboratory results should not be interpreted in isolation; it must be correlated with clinical information and be interpreted by registered medical practitioners only to determine final diagnosis.
7. Test results may vary based on time of collection, physiological condition of the patient, current medication or nutritional and dietary changes. Please consult your doctor or call us for any clarification.
8. Test results cannot be used for Medico legal purposes.
9. In case of queries please call customer care (91115 91115) within 48 hours of the report.

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